

EVIDENCE-INFORMED SUPPORT

The Little Guide

A minimalist, safety-first handbook to common
phytomedicine options for everyday complaints.

Evidence-informed guidance • everyday symptom support

START HERE

How to use this field guide

Keep the practical value. Lose the supplement hype.

What this handbook is

A practical educational guide to common phytomedicine options for everyday complaints. It is not a diagnosis, prescription, or a replacement for medical advice from a physician, pharmacist, dentist, midwife, psychologist, or other qualified health professional.

FOUNDATION Five rules before trying anything • Use one intervention at a time, not a stack. • Start low and stop if symptoms worsen. • Check interactions if you take any medicine. • Do not use supplements to delay urgent care. • Food, sleep, hydration, and movement remain the foundation.	USE WITH CARE Be skeptical of overpromising claims • Natural does not automatically mean safe. • Be cautious with products or posts that promise to cure disease. • Check interactions, quality, and the exact ingredient before trying anything. • If a claim sounds absolute or too good to be true, treat it skeptically.
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Important wording choice

This guide uses phrases like studied for, may support, and common supportive option. That is intentional. Evidence-informed does not mean proven for everyone, safe for everyone, or legal to advertise as a disease treatment.

Red flags - get professional care

- Severe, persistent, recurrent, or unexplained symptoms.
- Special-caution groups: pregnancy or breastfeeding, children, immune suppression, cancer therapy, anticoagulants, antidepressants, sedatives, or multiple prescriptions.

SAFETY CHECKLIST

The page to read before every chapter

Safety and restraint matter more than hype.

CHECK FIRST

Medication groups that need a pharmacist check

- Blood thinners / antiplatelets: warfarin, DOACs, aspirin, clopidogrel.
- Antidepressants, triptans, MAOIs, linezolid, or other serotonergic agents.
- Hormonal contraception, HIV therapy, cancer medicines, immunosuppressants.
- Diabetes, blood pressure, thyroid, heart rhythm, opioid, or sedative medicines.
- Any narrow-therapeutic-index drug: cyclosporine, tacrolimus, digoxin, lithium, antiepileptics.

LIMITS

Situations where natural is not enough

- Pregnancy or breastfeeding.
- Children and older adults with frailty.
- Upcoming surgery or dental surgery.
- Liver disease, kidney disease, bipolar disorder, psychosis, or autoimmune disease.
- A new symptom after starting any supplement.

Not included as a casual Danish recommendation: Ashwagandha

Ashwagandha is popular internationally, but it is not included here as a casual Danish recommendation. The Danish Veterinary and Food Administration states that foods containing ashwagandha are illegal to sell in Denmark following risk assessment concerns.

Your safe trial template

- Choose one option only.
- Use a product with a clear botanical name, part used, extract type, and strength.
- Use for a defined period, then reassess.
- Stop and seek advice if you notice rash, swelling, breathing symptoms, jaundice, dark urine, fainting, palpitations, severe GI symptoms, mood activation, or unusual bleeding.

QUICK INDEX

Pick the complaint, then check the safety box

This is a navigation page, not a diagnosis page.

How to use this index

Each row maps one everyday complaint to a primary phytomedicine option and a confidence rating. Turn to the numbered chapter for the full picture: what the evidence says, who it suits best, how it works in plain language, how to use it practically, and a red flags section listing when to seek professional care instead. A higher confidence rating does not mean safe for everyone - always check the safety column on the right before starting anything.

Everyday complaint	Primary option	Confidence	First safety thought
Travel nausea / queasy stomach	Ginger	Limited / mixed for motion sickness	Persistent symptoms or medicines: ask first
Period discomfort	Ginger + heat	Limited-moderate	Rule out severe / new pelvic pain
Gut cramps / bloating	Enteric peppermint oil	Moderate	Avoid with acid reflux / GERD (acid reflux disease)
Constipation / hard stools	Psyllium; senna rescue	Moderate	Water + obstruction red flags
Loose stools	Oral rehydration solution (ORS)	High for rehydration	Dehydration / blood / fever = care
Restless sleep	Valerian / lemon balm / lavender	Mixed to traditional	Do not combine with sedatives
Stress fatigue	Rhodiola	Traditional / limited	Avoid mania / serious anxiety
Joint comfort	Boswellia / curcumin	Limited / low certainty	Blood thinners / surgery caution
Low mood	St. John's wort / saffron	Positive evidence, but clinical supervision needed	Medication interactions first
Minor mouth irritation	Oral-safe aloe gel / specific licorice patches	Limited	Ulcers >2 weeks need exam

How to read confidence

High - established evidence, often from multiple trials or international guidelines (example: ORS for rehydration). **Moderate** - positive results in clinical trials but with some limitations in size, quality, or consistency. **Limited / traditional** - historically used but lacking strong clinical trial evidence; apply extra caution and do not rely on it for serious or persistent symptoms. **Emerging** - early-stage evidence that looks promising but is not yet robust enough for clinical recommendations. Even the strongest-confidence option in this guide still requires a safety and interaction check before use.

FIELD GUIDE CHAPTER

1. Ginger

Queasy stomach and nausea - evidence depends on the cause

EVIDENCE Evidence Limited and mixed for motion sickness. Most studies have not shown a clear benefit; evidence differs for other causes of nausea.	BEST FIT Best fit Adults with mild, short-lived nausea and no red flags who understand that benefit for motion sickness is uncertain.	CHECK FIRST Avoid / check first Pregnancy, any regular medicine, gallbladder disease, diabetes treatment, persistent symptoms, or severe vomiting.
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MECHANISM / CONTEXT Why it matters • Gingerols and shogaols have effects on gastrointestinal signalling in laboratory studies, but this does not prove clinical benefit for motion sickness. • Different studies use different extracts, powders, doses, and nausea settings, so results cannot be applied to every ginger product. • Foods, teas, candies, and standardized supplements are not interchangeable.	APPLICATION Practical use • Do not rely on ginger when nausea is severe, recurrent, unexplained, or accompanied by red flags. • Food or tea may be gentler than concentrated supplements, but it has not been proven to prevent motion sickness. • Avoid combining concentrated products or experimenting with high doses.
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Key takeaway Ginger is widely studied for nausea, but most motion-sickness studies have not shown clear benefit. Treat it as an optional comfort measure, not a reliable preventive treatment.

Red flags - get professional care • Repeated vomiting, dehydration, blood, severe abdominal pain, chest pain, head injury, pregnancy-related severe nausea, or symptoms lasting more than 24-48 hours. • Nausea after starting a new medicine or supplement.
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FIELD GUIDE CHAPTER

2. Ginger + heat

Period discomfort without replacing medical assessment

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Limited to moderate evidence for primary dysmenorrhea (period pain without an underlying structural cause), mostly from small and heterogeneous trials.	Predictable mild-to-moderate period discomfort in otherwise stable cycles.	Pregnancy, heavy bleeding, anticoagulants, severe or new pain, or endometriosis suspicion.

MECHANISM / CONTEXT What the evidence suggests	APPLICATION Practical use
• Small trials have compared ginger with anti-inflammatory painkillers (NSAIDs), but they do not establish equivalence to standard treatment. • Effects are most plausible when started early in the period window. • Heat therapy remains a low-risk non-herbal foundation.	• Use ginger as food, tea or a standardized product rather than random mega-doses. • Keep a cycle diary: pain score, bleeding, timing, and side effects. • Do not combine multiple pain supplements with NSAIDs (anti-inflammatory painkillers) without checking safety.

Key takeaway

Ginger's pungent compounds may reduce prostaglandin-driven pain signals (the chemical pathway responsible for menstrual cramps) and lower pain sensitivity, but the evidence is not strong enough to call it a guaranteed substitute for standard care.

Red flags - get professional care

- New, severe, one-sided, or rapidly worsening pelvic pain.
- Heavy bleeding, dizziness, fever, fainting, pregnancy possibility, pain between periods, pain with sex, or symptoms that disrupt life despite standard care.

FIELD GUIDE CHAPTER

3. Peppermint oil

Gut cramping, bloating, and IBS-type discomfort

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Moderate for short-term IBS (Irritable Bowel Syndrome) symptom improvement in adults; not a cure.	Adults with recurring gut cramps and bloating after serious causes have been considered.	Acid reflux / GERD (chronic acid reflux disease), hiatal hernia, gallbladder disease, liver disease, children, or pregnancy.

MECHANISM / CONTEXT Why enteric-coated matters	APPLICATION Practical use
- Laboratory evidence suggests that menthol can relax intestinal smooth muscle, including through effects on calcium channels. - Enteric-coated capsules are designed to delay release until after the stomach, which may reduce heartburn and improve tolerability. - Non-enteric peppermint oil may worsen heartburn.	- Use only products made for oral internal use. - Do not swallow essential oils from aromatherapy bottles. - Try a defined short period; stop if reflux, burning, rash, or breathing symptoms occur.

Key takeaway

Enteric-coated peppermint oil is a moderately supported option for short-term IBS-type gut discomfort in adults. The enteric coating is not optional - non-enteric formulations release menthol in the stomach and may worsen heartburn. Use only products formulated for internal oral use, not aromatherapy essential oils.

Red flags - get professional care

- Blood in stool, black stool, fever, persistent vomiting, unintentional weight loss, anemia, night-time diarrhea, new symptoms after age 50, severe pain, or a family history of bowel disease or cancer.

FIELD GUIDE CHAPTER

4. Psyllium + senna

Constipation, hard stools, and sluggish regularity

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Guidelines support a trial of fibre, particularly psyllium; evidence for senna is lower-certainty and product instructions matter.	Functional constipation with no obstruction red flags.	Swallowing problems, bowel obstruction, severe pain or vomiting, or chronic laxative need.

MECHANISM / CONTEXT	APPLICATION
Psyllium: slow and steady • Forms a gel with water, increasing stool water and bulk. • Must be taken with enough water to avoid choking or obstruction. • Separate it from medicines and supplements because fibre can reduce absorption.	Senna: rescue, not routine • A stimulant laxative commonly used for occasional constipation; follow the labelled dose and duration. • Repeated or prolonged use should be clinician-guided, particularly with kidney or heart disease, diuretics, or other medicines affected by fluid and electrolyte changes. • Add water, movement, regular toilet timing, and enough food volume.

Key takeaway

Increase fibre slowly to reduce gas and bloating. If laxatives are needed repeatedly, investigate the cause instead of normalising the pattern.

Red flags - get professional care

- New persistent constipation, especially after age 50.
- Blood, black stool, anemia, fever, vomiting, severe pain, unexplained weight loss, or alternating diarrhea and constipation.

FIELD GUIDE CHAPTER

5. ORS - Oral Rehydration Solution - first

Loose stools and dehydration risk

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
High for oral rehydration solution; phytomedicine is not first-line here.	Acute watery loose stools where hydration can be maintained and red flags are absent.	Prompt professional advice for children, frail older adults, pregnancy, immune suppression, blood in stool, high fever, or signs of dehydration.

MECHANISM / CONTEXT Why ORS matters most	APPLICATION What about berberine?
• The danger with loose stools is fluid and electrolyte (salt) loss, not just stool frequency - dehydration can become serious quickly. • ORS is a pharmacy sachet of salts and sugars dissolved in water; it works by using the body's sodium-glucose co-transport system (a gut mechanism that rapidly pulls water back into the bloodstream when salt and sugar are combined in the right ratio). • Pharmacy sachets should be mixed exactly as directed - too much or too little water reduces effectiveness.	• Berberine is not a substitute for oral rehydration or medical assessment. • Clinical benefit for routine self-treatment of acute diarrhoea is uncertain, while product quality, adverse effects, and drug interactions remain concerns. • Avoid self-treatment during pregnancy or breastfeeding and in infants or children; ask a pharmacist or doctor before use with any medicine.

Key takeaway

Start ORS early and continue normal food as tolerated. Avoid alcohol and very fatty meals during the acute phase. Think rehydrate first, suppress later only if appropriate.

Red flags - get professional care

- Dizziness, confusion, very little urine, inability to keep fluids down, blood or black stool, high fever, severe pain, persistent vomiting, diarrhea after antibiotics, or symptoms lasting more than 48-72 hours in adults.

FIELD GUIDE CHAPTER

6. Valerian, lemon balm & lavender

Restless sleep and mild stress overload

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Evidence depends on the preparation: EMA recognizes clinical evidence for a specific ethanolic dry extract, while several other preparations rely on traditional use. Valerian is not recommended by guidelines for chronic insomnia.	Short-term adult sleep disturbance linked to stress or routine disruption.	Sedatives, alcohol, opioids, pregnancy/ breastfeeding, liver disease, or bipolar/ mania risk.

MECHANISM / CONTEXT Evidence-weighted view	APPLICATION Practical use
- Valerian is widely used, but chronic insomnia guidelines do not recommend it as a treatment. - EMA notes that valerian has a gradual onset and is not suitable as an immediate treatment for acute sleep difficulty. - Lemon balm and lavender have traditional uses for mild stress-linked symptoms, but clinical evidence and formulations vary.	- Use sleep hygiene first: fixed wake time, morning light, caffeine cutoff, and low evening light. - Avoid driving or machinery if drowsy. - Use only for a defined short trial; stop with next-day dullness, agitation, or vivid dreams.

Key takeaway

These products may have calming or sedating effects, but mechanisms and clinical relevance vary by preparation. Sleep is also strongly shaped by behaviour, light exposure, stress, pain, and mental health.

Red flags - get professional care

- Insomnia for more than 3 months, loud snoring or breathing pauses, restless legs, panic, mania, depression, suicidal thoughts, alcohol-at-night pattern, or use of prescription sleep medication or opioids.

FIELD GUIDE CHAPTER

7. *Rhodiola rosea*

Stress-related fatigue and mental strain

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Traditional EU use plus limited clinical data; not a fix for unexplained fatigue.	Temporary stress fatigue after sleep debt or workload when serious causes are unlikely.	Bipolar disorder or mania history, significant anxiety or insomnia, pregnancy or breastfeeding, or use of antidepressant or other psychiatric medicines. Human interaction data are limited, so ask a pharmacist or doctor first.

MECHANISM / CONTEXT Why it is interesting	APPLICATION Practical use
• <i>Rhodiola</i> preparations contain compounds including rosavins and salidroside, but product composition varies. • EMA recognizes traditional use for temporary stress symptoms such as fatigue and weakness; this is not the same as proof from high-quality clinical trials. • Some users report stimulating effects, but robust safety and interaction data are limited.	• Morning or early afternoon only if it affects sleep. • Use a short defined trial, then reassess. • Stop if irritability, jitteriness, palpitations, anxiety, or insomnia worsen.

Key takeaway

Persistent fatigue can be caused by anemia, thyroid disease, sleep apnea, depression, infection, inflammatory disease, medication effects, overtraining, low energy intake, or chronic stress. Do not use a supplement to push through a problem that needs evaluation.

Red flags - get professional care

- Fatigue with weight loss, fever, night sweats, shortness of breath, chest pain, fainting, severe depression, suicidal thoughts, neurological symptoms, or recent medication changes.

FIELD GUIDE CHAPTER

8. Boswellia & curcumin

Joint comfort, stiffness, and mobility support

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Early positive findings for some osteoarthritis outcomes, but certainty is limited and product standardization matters.	Adult joint discomfort after diagnosis or serious causes have been considered.	Anticoagulants, surgery, pregnancy, gallbladder disease, liver disease, or cancer therapy.

MECHANISM / CONTEXT Boswellia	APPLICATION Curcumin/turmeric
- Laboratory studies suggest effects on 5-LOX and other inflammatory signalling pathways, but the clinically important mechanism in people remains uncertain. - Trials often use standardized extracts, not raw resin guesses. - Generally well tolerated in studies, but quality varies.	- Curcumin affects inflammatory signalling in laboratory studies, but clinical benefit has not been established definitively. - Enhanced bioavailability formulas can change exposure and interaction risk. - Highly bioavailable products have been linked to liver injury in some people; stop and seek advice for fatigue, nausea, dark urine, or jaundice.

Key takeaway

Think in terms of joint comfort and mobility support, not miracle claims. Product quality, interactions, and realistic expectations matter more than bold promises.

Red flags - get professional care

- Hot swollen joint, fever, injury, sudden severe pain, unexplained weight loss, worsening morning stiffness, neurological weakness, or symptoms suggesting inflammatory arthritis, gout, or infection.

FIELD GUIDE CHAPTER

9. St. John's wort & saffron

Low mood: evidence exists, but risk control comes first

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
St. John's wort has evidence for short-term mild-to-moderate depression, but study results vary and interaction risk is unusually high. Saffron trials are promising, but certainty and long-term safety remain limited.	Not for unsupervised treatment. Low mood warrants assessment when persistent, recurrent, worsening, or impairing daily life.	Any prescription medicine, hormonal contraception, bipolar or psychosis, pregnancy, or breastfeeding.

MECHANISM / CONTEXT	APPLICATION
St. John's wort - Speeds up liver enzymes (CYP3A4 and P-glycoprotein) that process many medicines, lowering their blood levels and reducing their effect - this is one of the most clinically significant herb-drug interactions known. - Dangerous when combined with antidepressants and other medicines that raise serotonin levels (serotonergic agents) - risk of serotonin syndrome. - Not for transplant, HIV, cancer therapy, anticoagulants, or hormonal contraception users without professional guidance.	Saffron - Meta-analyses report promising results for mood and anxiety symptoms, but many trials are small, short, and from a limited range of settings. - Fewer documented liver enzyme-induction interactions than St. John's wort, but saffron's full drug interaction profile has not been fully characterised - interaction data remain limited. - Avoid high doses, avoid during pregnancy, and do not combine with antidepressants unless supervised.

Key takeaway

Urgent care is required with suicidal thoughts, mania, psychosis, postpartum depression, or severe functional decline. This chapter exists to show risk-aware positioning, not to normalise self-treatment of serious depression.

Red flags - get professional care

- Suicidal thoughts, self-harm thoughts, psychosis, mania, postpartum depression, severe functional decline, substance-use crisis, trauma-linked symptoms, or recurrent depression.

FIELD GUIDE CHAPTER

10. Aloe & licorice patches

Minor mouth irritation and aphthous ulcer (canker sore) comfort

EVIDENCE Evidence	BEST FIT Best fit	CHECK FIRST Avoid / check first
Limited and product-specific topical evidence; not a cure or a substitute for oral-cancer assessment.	Small recurrent aphthous ulcers (canker sores - common painful white or grey mouth ulcers) or local irritation after obvious triggers such as spicy food or minor trauma.	Ulcers lasting more than 2 weeks, hard or raised edges, immune suppression, chemotherapy, or systemic symptoms.

MECHANISM / CONTEXT Aloe vera gel	APPLICATION Licorice/glycyrrhiza patches
• Use only oral-safe products intended for use on the mouth lining (oral mucosa) - not all aloe products are safe to put in the mouth. • Small studies suggest possible benefit for selected oral conditions, but evidence for ordinary aphthous ulcers is not robust. • Avoid aloe latex laxative products in this context.	• Licorice patch formulations vary; do not assume a product is deglycyrrhizinated (DGL) or free of glycyrrhizin unless the label says so. • One small trial suggests that a specific dissolving licorice patch may reduce aphthous-ulcer pain; this cannot be generalized to every patch or lozenge. • Swallowed glycyrrhizin-containing licorice can raise blood pressure and lower potassium. Use only products intended for the mouth and follow their instructions.

Key takeaway

The point is not natural cures. It is thoughtful, evidence-weighted phytomedicine with safety first. A good guide should build trust, not overpromise.

Red flags - get professional care

- Ulcer lasting longer than 2 weeks, growth, easy bleeding, hard edges, fever, weight loss, swollen lymph nodes, genital ulcers, eye pain, chronic diarrhea, anemia, joint swelling, or immune suppression.

REFERENCES

Core sources and further reading

Enough to support the evidence-weighted wording - without making the guide feel academic and heavy.

Reference note

References are included to support the evidence-informed language in this handbook. They are not a substitute for product-specific legal review, professional medical care, or personalised advice.

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